INTERNSHIP REPORT

Submitted

For the Partial Fulfilment of the Requirement for the Degree of Bachelor of _____ with _____ (Major and Minor Subjects) under **Magadh University**

Submitted to

The Department of _____, _____ **(College Name)**

[Paste College Logo here]

Submitted by

Name : _____

Semester : _____

Programme FYUGP : _____

University's Registration No. : _____

College Roll No. : _____

Year : _____

RK NEXORA
CAMPUS TO CAREER
A Unit of RKNexora Private Limited.

INTERNSHIP REPORT

Topic - Hospital & Healthcare Administration

Submitted to the Head of Department, _____ (College Name), in partial fulfilment of the requirements for the Degree of _____, Magadh University

Name of Student : _____

Programme : _____

University Registration No. : _____

College Roll No. : _____

Internship Registration No. : _____

Name of the IPO / Agency : RKNEXORA PRIVATE LIMITED

Internship Period : _____ to _____

Semester : _____

Year / Session : _____

RKNEXORA PRIVATE LIMITED
RKNEXORA
PRIVATE LIMITED
GAYA, BIHAR - 805236, INDIA

_____ _____

Signature of Student **Seal & Signature of IPO**

ACKNOWLEDGEMENT

I express my sincere gratitude to RK NEXORA, RKNEXORA PRIVATE LIMITED, _____ (College Name), my faculty mentors and the internship supervisor, _____, for their guidance and support. The structured activities, learning resources and regular feedback helped me complete this internship in Hospital & Healthcare Administration successfully.

I am thankful to the Head of the Department and the college mentor for their encouragement and for guiding me in preparing this report. I am also grateful to the management of RKNEXORA PRIVATE LIMITED for providing a professional learning environment and the opportunity to learn and apply my knowledge in a practical setting. The concept sessions, hands-on tasks with sample data and case studies, and the supervisor's patient and detailed feedback helped me understand how a hospital is organised, how patients, records, billing and quality are managed, and how patient safety is protected.

Finally, I thank my family and peers for their constant encouragement throughout the programme.

_____

Name: _____

University Registration No. - _____

INTERNSHIP COMPLETION CERTIFICATE

The Internship Completion Certificate issued by RKNEXORA PRIVATE LIMITED for the internship in Hospital & Healthcare Administration is to be attached on this page.

[Attach the original / printed copy of the QR-verified Internship Completion Certificate here]

Certificate No. _____

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PREFACE

Internships are an important part of the undergraduate programme, as they allow students to see how classroom learning is applied in real organisations. The Four-Year Undergraduate Programme (FYUGP), introduced in line with the National Education Policy 2020, gives special importance to experiential learning, skill development and employability. This report has been prepared as part of the internship requirement of Magadh University and the Higher Education guidelines applicable in Bihar.

The report describes the internship undertaken in Hospital & Healthcare Administration at RKNEXORA PRIVATE LIMITED, the plan followed, the work carried out, and the learning gained. A hospital is one of the most complex organisations in society. Doctors and nurses treat patients, but the treatment works only when many other things work: the patient is registered correctly, the bed and the operation theatre are ready, the medicines are in stock, the records are complete and safe, the bill and the insurance claim are correct, and the complaint of a worried relative is heard. Hospital administration is the work of organising all of these so that care is safe, timely and affordable. It offers many careers and demands both accuracy and sensitivity to people in difficult moments. This report describes how the main areas of hospital administration were studied and how a workflow and patient records management study was carried out for a fictitious 100-bed hospital using dummy data. No real patient data was used at any time.

The report has been arranged in the order suggested by the University, beginning with the cover page and acknowledgement and ending with the conclusion and attendance sheet. It is hoped that it gives a clear and honest account of the internship experience. All examples in this report use dummy data and fictitious names; no real personal or business information has been handled.

EXECUTIVE SUMMARY

This report documents the internship completed in Hospital & Healthcare Administration at RKNEXORA PRIVATE LIMITED (RK NEXORA - 'Campus to Career'). The programme required a minimum of 120 hours, completed over about four to five weeks (28 working days) with daily sessions of four to five hours, and was supported by an attendance log, supervisor guidance and review.

Background. Hospital administration is the planning, organising, directing and controlling of the resources and services of a hospital or healthcare organisation. Its areas include patient registration and records, outpatient (OPD) and inpatient (IPD) management, billing and insurance claims, medical records (MRD), inventory and pharmacy, quality and patient safety, accreditation standards such as NABH, healthcare laws and ethics, and the handling of patient feedback and grievances. The aim is to support the clinical team and to give every patient safe and respectful care.

Objectives. The internship aimed to (i) understand the structure and workflow of a hospital, (ii) follow the steps of patient registration and record keeping, (iii) understand OPD and IPD management, (iv) understand hospital billing and insurance claims, (v) understand medical records management, (vi) understand pharmacy and inventory control, (vii) understand quality and patient safety, (viii) understand NABH standards, (ix) understand healthcare laws and ethics, and (x) handle patient feedback and grievances.

Work carried out. Each topic had a concept session followed by hands-on practice with dummy forms and data of a fictitious hospital, Shree Sanjeevani Hospital, Gaya. The student traced the patient journey, filled dummy registration and admission forms, prepared an OPD schedule, calculated a bill and an insurance claim, designed a medical record filing system, calculated a reorder level for medicines, prepared an infection-control audit, studied the NABH chapters and handled sample grievances. The last part of the programme was devoted to the project, a case study exercise, a mock practical exercise, supervisor review and preparation of this report.

Project. The work was tied together in a project titled **Hospital Workflow & Patient Records Management**. The project mapped the patient workflow and the records process of a fictitious 100-bed hospital, calculated its main indicators (bed occupancy 75%, average length of stay 5.0 days), worked out a bill and an insurance claim, and prepared an improvement plan for waiting time and record completeness.

Method. A blended method was followed - learn, practise, document and review - supported by a daily attendance log with day-wise remarks and regular feedback from the supervisor.

Learning gained. The student gained a clear understanding of the structure of a hospital and the patient journey, practical skill in registration, OPD and IPD processes, billing and claims, records, inventory control, quality and safety, NABH standards, healthcare laws and ethics, and in handling feedback and grievances with courtesy.

Key observations. Many problems in hospitals come from poor coordination between departments and not from lack of medical skill; accurate records protect both the patient and the hospital; patients value clear information and respect as much as speed; and small safety habits, such as hand hygiene and correct identification, prevent serious harm.

Conclusion. The internship met its objectives and contributed to the student's academic understanding, employability and confidence. The report follows the University's suggestive format: cover page, acknowledgement, certificate, contents, preface, executive summary, about the agency, plan and execution, learning outcome, conclusion and attendance sheet.

ABOUT THE AGENCY

RK NEXORA, a unit of RKNEXORA PRIVATE LIMITED, provides structured internship and skill-development programmes under the theme 'Campus to Career'. The organization focuses on academic-industry alignment, practical exposure, assessment, documentation and employability-oriented learning.

Particulars	Details
Name of Agency	RKNEXORA PRIVATE LIMITED
Programme / Unit	RK NEXORA - Campus to Career
Registered Office	C/O Mukesh Kumar, Khiri, PS Atri, Jethiyan, Atri, Gaya - 805236, Bihar, India
CIN	U85499BR2026PTC086757
Email	rknexorapvtltd@gmail.com
Website	www.rknexora.org
Internship Topic	Hospital & Healthcare Administration

1. Vision

To support students in developing practical, professional and industry-relevant capabilities.

2. Mission

To deliver structured learning experiences that connect academic knowledge with workplace expectations.

3. Activities of the Agency

1. Structured internship programmes aligned with university and higher-education requirements.
2. Skill-development learning modules, practical activities and assignments.
3. Attendance tracking, supervision, assessment and documentation.
4. Employability-oriented guidance connecting campus learning to career readiness.

4. Programme Structure

The internship programme is organised around a fixed number of learning hours, completed over a defined period. Each intern is registered on the internship portal with a unique registration number, is assigned a supervisor, follows a day-wise plan and maintains an attendance log. Learning is organised into three phases: **Orientation** (Days 1-2), **Learning and Practice** (Days 3-22, ten sub-topics of two days each: one concept day and one hands-on day) and **Project and Review** (Days 23-28), followed by a concluding review.

5. Role of the Supervisor

The internship supervisor assigned by the agency guided the student through the programme, explained concepts, assigned practice tasks, reviewed the daily log and gave feedback. The supervisor also verified attendance and reviewed the student's work at the end of the programme.

PLAN AND EXECUTION

1. Objectives

The internship was planned around the following objectives, each linked to a part of the programme:

1. Understand hospital departments and the patient workflow (Week 1).
2. Understand patient registration and record keeping (Week 1).
3. Understand OPD, IPD, billing and insurance claims (Week 2).
4. Understand medical records, pharmacy and inventory (Week 2-3).
5. Understand quality, patient safety and NABH standards (Week 3).
6. Understand healthcare laws, ethics, feedback and grievances (Week 3-4).
7. Complete the project, case study and report (Weeks 4-5).

2. Methodology

The programme followed a blended method made up of four connected steps, repeated for each topic:

Learn. Each topic began with a concept session in which the supervisor explained the idea, its main terms and how it is used in real organisations. Notes were prepared in the student's own words.

Practise. The concepts were then applied in guided hands-on tasks - drawing the patient journey, filling dummy registration forms, preparing OPD and bed charts, calculating a bill and an insurance claim, designing a record filing system, calculating a medicine reorder level, preparing an audit sheet and handling dummy complaints. Only sample data and fictitious examples were used, so that no real personal or business information was handled.

Document. Every day's work was recorded in the internship attendance log with a short remark, and sheets, files and screenshots were saved so that the work could be reviewed.

Review. The supervisor reviewed the work, pointed out errors and suggested improvements. Review days, a case study and a mock practical exercise helped to consolidate the learning.

3. Understanding Hospital & Healthcare Administration - Concepts Studied

3.1 Hospital departments and workflow

A hospital has clinical departments, which give direct care (OPD, wards, operation theatres, ICU, emergency, radiology and laboratory), and support and administrative departments, which keep the clinical work running (front office, billing, medical records, pharmacy and store, housekeeping, central sterile supply, human resources, finance, information technology and quality). The Medical Superintendent or Hospital Administrator coordinates them. A hospital is classified by size (the number of beds), by ownership (government, private, trust), and by services (general or speciality). The structure of the dummy hospital is shown below.

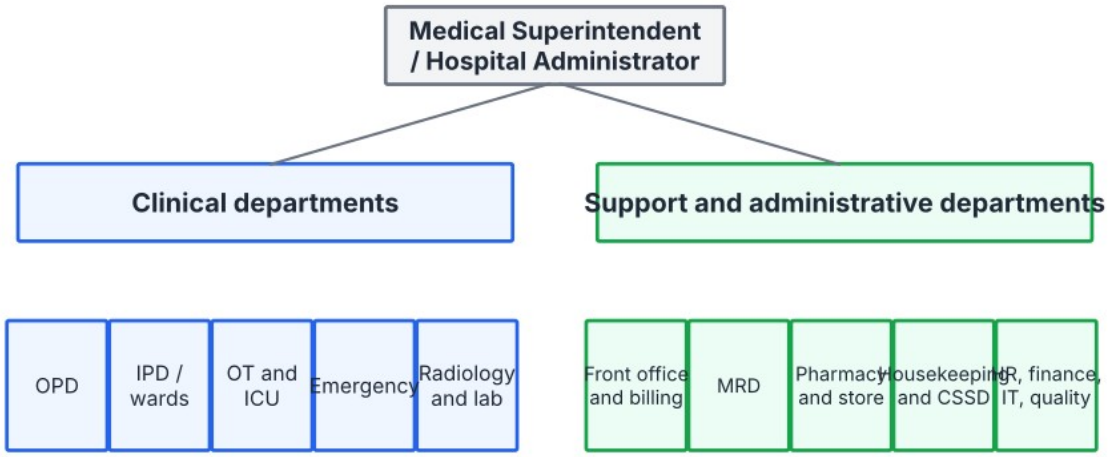


Figure 1: Departments of a hospital

The patient journey connects these departments. A patient registers at the front office, is seen in the OPD, may be advised tests (laboratory, radiology) and medicines (pharmacy), and, if needed, is admitted, treated in the ward or operation theatre, billed and discharged, and invited for follow-up and feedback. A delay or an error at one point - an incomplete form, a missing report, a bed not ready - delays the whole chain, which is why coordination is the main task of the administrator.

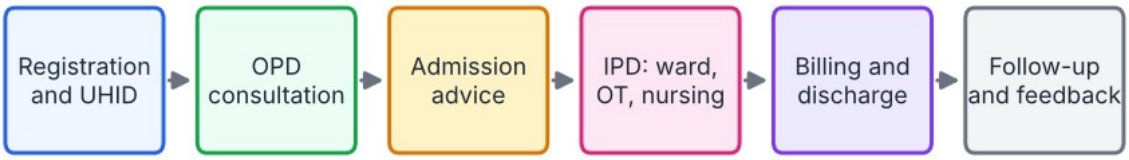


Figure 2: The patient journey

3.2 Patient registration and records

Registration creates the identity of the patient in the hospital's system. A new patient is given a Unique Health ID (UHID) and a registration card; the details taken are the name, age, sex, address, phone number, identity proof where needed, the person to be contacted in an emergency, and consent to treatment. For a returning patient the UHID is searched to avoid duplicate records. The steps are to ask the patient or the attendant politely, search for an existing record, enter the data accurately, check it by reading it back, issue the card and send the patient to the right counter. In India the Ayushman Bharat Digital Mission offers a health account number called ABHA, which a patient may choose to link to the records with consent. The student practised filling dummy registration forms, of which an extract is shown below.

> **Extract of a registration form (dummy data)**
> Patient registration form (dummy) - Shree Sanjeevani Hospital, Gaya
> UHID: SSH-000127 Date: 12/09/2026 Time: 09:40
> Name: (dummy patient) Age/Sex: 34 / F Phone: XXXXXXXXXX
> Address: (dummy address), Gaya Emergency contact: (dummy name)
> Visit type: [x] OPD [ ] Emergency [ ] Admission Department: General Medicine
> Payer: [ ] Self [x] Insurance / TPA (card no. _____) [ ] PM-JAY
> Consent for treatment and data use: [x] Given Signature / thumb impression: _____

Common errors are spelling mistakes, wrong age, wrong phone number and duplicate UHIDs; they cause problems later in billing and records. A simple control is to ask the patient to confirm the name and phone number once more. Patient identity must be verified again at every critical step, such as before a procedure or a transfusion, using two identifiers such as name and UHID.

3.3 OPD and IPD management

The Outpatient Department (OPD) serves patients who do not need to stay. The administrator plans doctor timings, token or appointment systems, consultation rooms, waiting areas and the flow to the laboratory and pharmacy. An appointment system reduces crowding, and tokens keep the order fair. The Inpatient Department (IPD) manages patients who are admitted: the admission advice, the admission form and consent, the allotment of a bed according to the entitlement and the clinical need, the shifting to the ward, daily rounds and nursing care, investigations and procedures, the discharge summary, billing and the discharge itself. The key indicators of IPD performance are given below, with the figures of the dummy hospital for one month of 30 days.

Indicator	Formula	Calculation	Result
Bed occupancy rate (BOR)	Inpatient days / (beds x days) x 100	2,250 / (100 x 30) x 100	75%
Average length of stay (ALOS)	Inpatient days / discharges	2,250 / 450	5.0 days
Bed turnover rate	Discharges / beds	450 / 100	4.5 patients per bed
Turnover interval	(Available bed days - inpatient days) / discharges	(3,000 - 2,250) / 450	1.67 days

A BOR of 75% means that three-quarters of the bed-days were used. A very low BOR means wasted capacity, and a very high BOR (above about 85-90%) means crowding, a delay in admission and little time for cleaning and preparing the beds. The turnover interval shows how long a bed remains empty between patients; a long interval points to delays in discharge, cleaning or admission. The OPD load of a typical week for the dummy hospital is shown below; Monday is the busiest day and Saturday the lightest, which suggests that extra counters and doctors should be placed on Mondays.

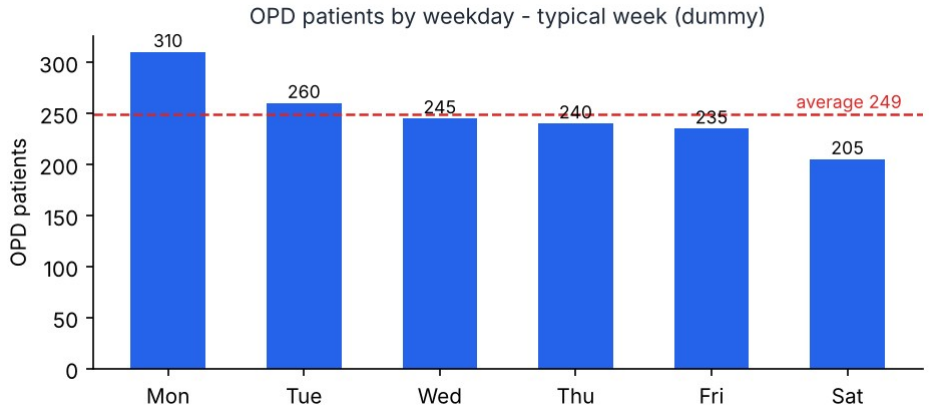


Figure 3: OPD patients by weekday in a typical week

3.4 Billing and insurance claims

Hospital billing records the services given and the charges for them. A bill is built from the room or bed charges, doctor and nursing charges, procedure or surgery charges or a package rate, investigations, medicines and consumables, and other services. Charges follow the hospital's approved rate list, and a package rate covers a defined set of services for a fixed price. The bill must be accurate, itemised and explainable, since it is often the most sensitive moment for the family. A patient may pay in cash, or the cost may be paid wholly or partly by health insurance, a government scheme such as Ayushman Bharat PM-JAY (which gives cashless cover to eligible families, up to a limit per family per year) or an employer. A Third Party Administrator (TPA) processes insurance claims for many insurers. In a cashless claim, the hospital sends a pre-authorisation request before the planned treatment, the insurer approves an amount, the hospital treats the patient and submits the final claim with the documents (the discharge summary, bills, investigation reports and identity and policy papers), and the insurer settles the claim, usually after questions. The steps are shown below.

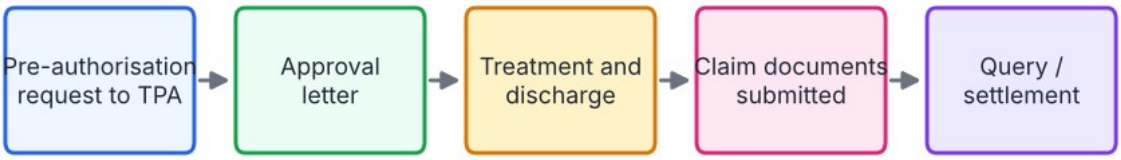


Figure 4: Steps of a cashless insurance claim

Worked example. A dummy patient is admitted for a planned surgery under a policy with a sum insured of Rs. 1,00,000 and a co-payment of 10% on admissible expenses. The itemised bill is shown below.

Item	Amount (Rs.)	Admissible?
Surgery package (OT, surgeon, anaesthesia and 3 days' room)	45,000	Yes
Extra room stay, 1 day	2,500	Yes
Medicines	12,500	Yes

Item	Amount (Rs.)	Admissible?
Investigations (lab and radiology)	6,800	Yes
Consumables not covered by the policy	4,200	No
Total bill	71,000	

The admissible amount is 71,000 - 4,200 = Rs. 66,800. The co-payment is 10% of 66,800 = Rs. 6,680, so the insurer pays 66,800 - 6,680 = Rs. 60,120. The patient pays the co-payment and the non-admissible items: 6,680 + 4,200 = Rs. 10,880. The total, 60,120 + 10,880 = Rs. 71,000, agrees with the bill. The figure below shows the division. Claims are often delayed or reduced because of missing documents, wrong diagnosis codes, services not covered or approval limits exceeded, so a checklist of documents before submission saves much time.

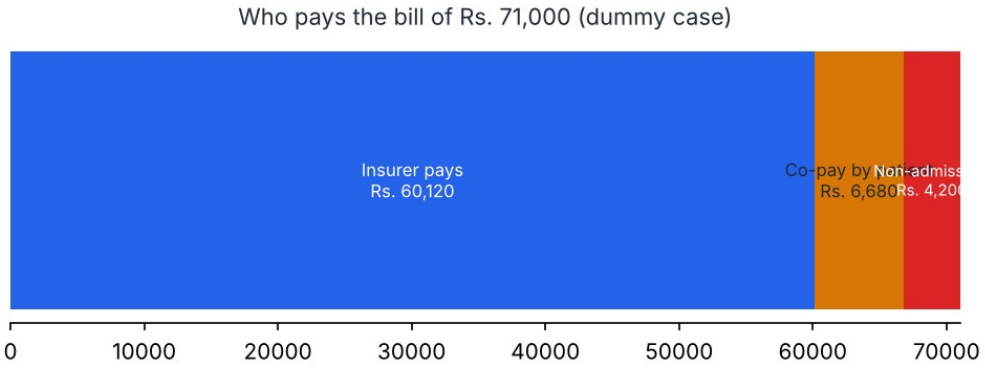


Figure 5: Division of the bill between the insurer and the patient

3.5 Medical records (MRD)

The medical record is the written or electronic history of the patient's illness and care: the registration data, the consent forms, the clinical notes, the investigation reports, the nursing charts, the operation notes, the discharge summary and the bills. It supports the continuity of care, the legal protection of the patient and the hospital, audits, research and claims. The Medical Records Department (MRD) receives, checks, codes, files, stores, retrieves and, after the required period, destroys the records according to the rules. Files are arranged by UHID, often in terminal digit filing, in which the last digits of the number decide the shelf, so that files spread evenly and retrieval is quick. Records are confidential: they are given only to the patient, an authorised representative, the treating team or, as required by law, to authorities, and every issue is logged. The Indian Medical Council (Professional Conduct, Etiquette and Ethics) Regulations, 2002 require physicians to maintain indoor patient records for three years from the commencement of treatment; the hospital's policy and the legal advice should be checked for other periods. Electronic records bring the benefits of quick search and backup, and the duties of access control, audit trails and data protection under the Digital Personal Data Protection Act, 2023. A record completeness audit checks that each file has the required forms, signatures, dates and times.

Audit item (dummy sample of 50 discharge files)	Files complete	Completeness
Consent form signed	47 of 50	94%
Discharge summary present	45 of 50	90%
Doctor's notes dated and signed	42 of 50	84%
Investigation reports filed	48 of 50	96%

The weakest item is the dating and signing of the doctor's notes (84%), so a reminder and a signature check before the file is sent to the MRD were suggested.

3.6 Hospital inventory and pharmacy

A hospital must always have medicines, consumables and equipment in the right quantity: too little endangers patients, and too much blocks money and leads to expiry. The pharmacy and the store follow the cycle of indenting, purchase, receipt and inspection, storage, issue and accounting. Important controls are the ABC analysis (A items are few but of high value and need strict control), the FEFO rule (first expiry, first out), the expiry tracking, the storage at the correct temperature, the register of controlled drugs, and the regular physical verification of stock. The reorder level is the stock at which a new order is placed: reorder level = (average daily use x lead time) + safety stock. For a medicine used at 200 tablets a day, with a supplier lead time of 5 days and a safety stock of 300 tablets, the reorder level is (200 x 5) + 300 = 1,300 tablets. When the stock falls to 1,300 tablets, an order is placed, so that the new stock arrives before the safety stock is used. Medicines near expiry are reported to the doctors for early use or returned to the supplier, and expired medicines are discarded by the rules of biomedical and pharmaceutical waste.

3.7 Quality and patient safety

Quality in healthcare means care that is safe, effective, timely, patient-centred, efficient and equitable. Patient safety is the prevention of harm to patients from the care itself. The main practices are correct patient identification with two identifiers, hand hygiene, safe medication practices (the right patient, drug, dose, route and time), safe surgery checklists, prevention of falls and pressure sores, infection control with sterilisation and biomedical waste segregation, clear handover of care, and open reporting of incidents and near misses without blaming individuals. Quality is measured by indicators such as the infection rates, the waiting time, the readmission rate and the patient satisfaction. For example, a hand hygiene audit counts the opportunities observed and the number when hand hygiene was done: of 200 opportunities, 172 were compliant, a compliance of 172 / 200 = 86%, against a target of 95%. A catheter-associated urinary infection rate is calculated as the number of infections / catheter days x 1,000; three infections in 1,200 catheter days give 2.5 per 1,000 catheter days.

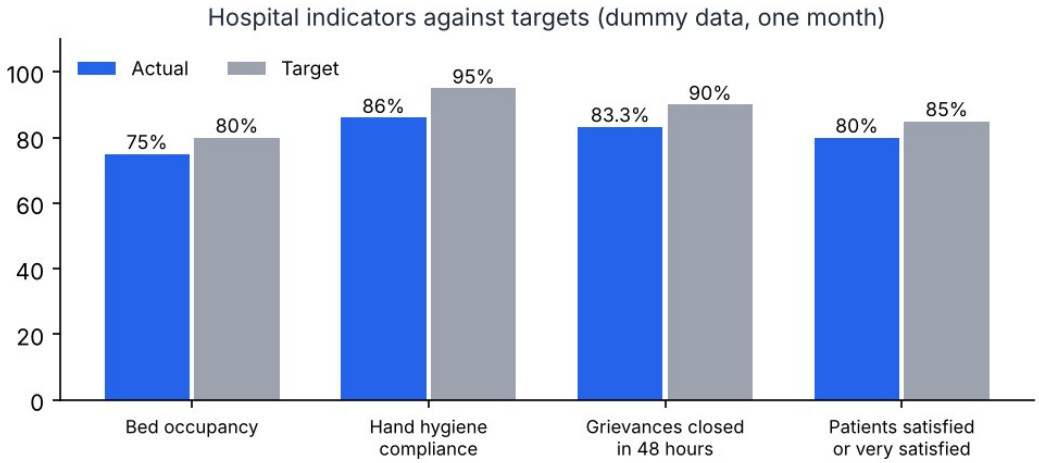


Figure 6: Hospital indicators against targets (dummy data)

All four indicators fall below their targets, and the largest gap is in hand hygiene (86% against 95%), which is the cheapest and most effective measure against hospital infections. Corrective actions are training, placing hand rub at every bed, and monthly feedback to the wards.

3.8 NABH standards

The National Accreditation Board for Hospitals and Healthcare Providers (NABH), a constituent board of the Quality Council of India, gives accreditation to hospitals that meet its standards, which improves patient safety and public trust. Its standards for hospitals are arranged in chapters, including Access, Assessment and Continuity of Care (AAC); Care of Patients (COP); Management of Medication (MOM); Patient Rights and Education (PRE); Hospital Infection Control (HIC); Patient Safety and Quality Improvement (PSQ); Responsibilities of Management (ROM); Facility Management and Safety (FMS); Human Resource Management (HRM); and Information Management System (IMS). The edition and the details change from time to time, so the latest standards must be studied from the NABH website. Preparation for accreditation involves a gap analysis, written policies and procedures, training of the staff, mock audits and a continuing record of evidence. A key lesson is that a standard is met only when it is written, known to the staff and followed in practice. The student prepared a simple readiness checklist for three chapters, shown in section 13.

3.9 Healthcare laws and ethics

Hospitals work within a framework of laws and ethics. The laws that an administrator should know include the Clinical Establishments (Registration and Regulation) Act, 2010 in the states that have adopted it, the Consumer Protection Act, 2019 under which patients may seek a remedy for deficiency in service, the Bio-Medical Waste Management Rules, 2016, the Pre-Conception and Pre-Natal Diagnostic Techniques (PCPNDT) Act, 1994, the Mental Healthcare Act, 2017, the Medical Termination of Pregnancy Act as amended, the Digital Personal Data Protection Act, 2023 and local rules for fire, licences and waste. The ethical principles are respect for the autonomy of the patient (informed consent), beneficence (doing good), non-maleficence (avoiding harm), justice (fair treatment of all) and confidentiality. Patient rights include the right to information, to respect and dignity, to privacy, to refuse treatment, to a second opinion, to a clear bill and to make a complaint. Informed consent is obtained in a language the patient understands, with the risks and alternatives explained, and is recorded before any procedure; for minors and persons unable to consent, a legal guardian gives it. Emergency care must not be delayed for want of payment or paperwork. The student noted that the laws and rules change, and that the legal officer or adviser must be consulted for any particular case.

3.10 Patient feedback and grievances

Feedback from patients is a cheap and powerful guide to quality. It may be taken on a short form, at the bedside, by phone after discharge or through a QR code. The questions cover waiting time, behaviour of staff, cleanliness, communication of the doctor, food and the billing. A grievance is a complaint, and the aim is to treat it with respect, resolve it quickly and learn from it. The steps are to receive the complaint politely, log it with a number, acknowledge it, investigate with the department concerned, take action and reply to the patient, close the case and analyse the complaints each month for repeated causes. The dummy hospital's feedback for one month is shown below.



Figure 7: Steps of grievance handling

Measure	Result	Calculation
Feedback forms received	120	
Very satisfied / satisfied	54 / 42	(54 + 42) / 120 = 80%

Measure	Result	Calculation
Neutral	14	11.7%
Dissatisfied / very dissatisfied	6 / 4	8.3% together
Complaints received	18	
Closed within 48 hours	15	15 / 18 = 83.3%

The main complaints were about waiting time at the billing counter and discharge delays, which led to the improvement plan in the project. A grievance should never be treated as an attack: a patient who is heard calmly often becomes a supporter of the hospital.

4. Week-wise Plan and Execution

Week	Days	Activities
Week 1	Days 1-6	Orientation and foundations: company induction, introduction to hospital administration, hospital departments and workflow, and patient registration and records.
Week 2	Days 7-12	Patient services: OPD and IPD management, billing and insurance claims, and medical records (MRD).
Week 3	Days 13-18	Support and quality: hospital inventory and pharmacy, quality and patient safety, and NABH standards.
Week 4	Days 19-24	Application: healthcare laws and ethics, patient feedback and grievances, the Hospital Workflow & Patient Records Management project, a case study exercise and a mock practical exercise.
Week 5	Days 25-28	Project closure: supervisor review and feedback, internship report drafting, presentation preparation and final review.

5. Week-wise Narrative

Week 1: The first week was devoted to orientation and foundations. After company induction, the student studied the departments of a hospital, traced the patient journey from registration to follow-up and practised filling dummy registration forms.

Week 2: In the second week the student studied OPD and IPD management and calculated bed occupancy and average length of stay, prepared an itemised bill and an insurance claim and studied the filing and confidentiality of medical records.

Week 3: The third week was devoted to the control of medicines and consumables with reorder levels, to patient safety and infection control audits and to the NABH chapters.

Week 4: In the fourth week the student studied healthcare laws, patient rights and consent, handled dummy grievances and completed the project, a case study exercise and a mock practical exercise.

Week 5: The last week was devoted to project closure: supervisor review and feedback, drafting of the internship report, preparation of a presentation and final review before submission.

6. Topic-wise Learning in Detail

6.1 Hospital Departments and Workflow

Concept: Clinical and support departments, patient flow from entry to discharge and coordination between departments. **Practice carried out:** Drawing the department structure and the patient journey (Figures 1 and 2).

6.2 Patient Registration and Records

Concept: Patient registration steps, UHID creation, admission forms and record keeping. **Practice carried out:** Filling dummy registration and admission forms and checking for duplicates.

6.3 OPD / IPD Management

Concept: OPD appointments, consultation flow, IPD admission, bed allotment and discharge process. **Practice carried out:** Preparing a doctor-wise OPD schedule and calculating BOR and ALOS (section 3.3).

6.4 Billing and Insurance Claims

Concept: Hospital billing, package rates, TPA/insurance pre-authorisation and claim documents. **Practice carried out:** Preparing an itemised bill and a claim with a co-payment (section 3.4).

6.5 Medical Records (MRD)

Concept: Filing, retrieval, storage and confidentiality of medical records. **Practice carried out:** Designing a terminal digit filing plan and a record completeness audit (section 3.5).

6.6 Hospital Inventory and Pharmacy

Concept: Stock of medicines and consumables, issue and receipt records, expiry and reorder tracking. **Practice carried out:** Calculating a reorder level and preparing an expiry list (section 3.6).

6.7 Quality and Patient Safety

Concept: Patient safety protocols, infection control, incident reporting and quality indicators. **Practice carried out:** Preparing a hand hygiene audit and a catheter infection rate (section 3.7).

6.8 NABH Standards

Concept: NABH chapters, documentation requirements and audit preparation. **Practice carried out:** Preparing a readiness checklist for three chapters.

6.9 Healthcare Laws and Ethics

Concept: Healthcare regulations, patient rights, consent and medical ethics. **Practice carried out:** Preparing a consent form checklist and a patient rights notice.

6.10 Patient Feedback and Grievances

Concept: Collecting feedback, handling complaints and closing grievances. **Practice carried out:** Analysing 120 feedback forms and handling five dummy complaints (section 3.10).

7. Ethics, Confidentiality and Patient Rights - Points Studied

Hospital staff handle the health, the money and the private information of people in difficult situations, so ethics are part of every task. The points studied are summarised below.

- **Confidentiality:** patient information is shared only with the treating team and authorised persons; never discuss a patient in public places or on social media.
- **Informed consent:** explain in simple words and obtain consent before procedures and before the use of data.
- **Dignity and non-discrimination:** treat every patient with the same respect, regardless of money, religion, caste or gender.

- **Honesty in billing:** charge only for services given and at the approved rates; no hidden or false claims.
- **Emergency care:** never delay emergency care for want of money or papers.
- **Safety first:** report errors and near misses; a safe system learns from them.
- **Data protection:** use passwords, log out of systems and do not copy records; the Digital Personal Data Protection Act, 2023 applies. Only dummy data was used in the internship.

8. Process Flows Studied

8.1 Steps of an IPD admission

- Receive the admission advice and verify the patient's identity and UHID.
- Explain the charges, take the consent and complete the admission form.
- Check the insurance or scheme details and send the pre-authorisation if cashless.
- Allot the bed and inform the ward and the pharmacy.
- Open the record file and start the daily charge entries.

8.2 Steps of a discharge

- Doctor writes the discharge order and the discharge summary.
- The ward completes the medicine reconciliation and nursing notes.
- Billing prepares the final bill and checks the insurance approval.
- Pharmacy issues the discharge medicines; the patient is counselled.
- Records are completed, sent to the MRD and the bed is released for cleaning.

9. Worked Examples with Sample (Dummy) Data

Worked example 1 - finding the bottleneck in discharge. In the dummy hospital the time from the doctor's discharge order to the patient leaving was timed for ten patients, and the average was 4 hours 10 minutes. The steps averaged: discharge summary 55 minutes, nursing and medicine reconciliation 35 minutes, final bill preparation 70 minutes, insurance approval of the final bill 60 minutes (for cashless patients) and pharmacy and counselling 30 minutes. The longest steps are the insurance approval and the bill preparation, which together take 130 of the 250 minutes. Starting the bill as soon as the discharge is planned, and sending the interim bill to the TPA earlier, could save about an hour. The bed is therefore released earlier, which also raises the bed turnover.

Worked example 2 - effect on bed turnover. If the discharge is made one hour faster for 450 discharges a month, 450 bed-hours (about 18.75 bed-days) are released. At an ALOS of 5.0 days, each new patient uses 5 bed-days, so this is the capacity of roughly 3 to 4 more patients a month, without any new bed. The figure is small, but the saving also reduces the crowding in the evening, the stress of the family and the load on the housekeeping.

Worked example 3 - cost of a missing document. A claim for Rs. 60,120 was returned by the TPA because the discharge summary did not state the diagnosis in the form required. The correction took nine days. If the payment is delayed by nine days, the hospital's working capital is blocked for that time; at an assumed cost of money of 12% a year, the cost of the delay is 60,120 x 0.12 x 9 / 365 = about Rs. 178, besides the extra work and the displeasure of the patient. This shows that a documents checklist before submission is cheap and useful.

10. Project: Hospital Workflow & Patient Records Management

Problem statement. The administrator of Shree Sanjeevani Hospital, Gaya, a fictitious 100-bed hospital, wants to understand the patient workflow and the records process, to measure its main indicators, and to reduce the waiting time at billing and the delay in discharge. All the data is dummy data prepared for learning; no real patient information was used.

10.1 Method

- Map the patient journey and time each step for a sample of dummy patients.
- Calculate the IPD indicators (BOR, ALOS, turnover) from the monthly figures.
- Audit the completeness of 50 dummy discharge files and the hand hygiene compliance.
- Analyse the feedback forms and the complaints of the month.
- Prepare an improvement plan with responsible persons and dates.

10.2 Key findings

Area	Finding
Occupancy	BOR 75%, ALOS 5.0 days, bed turnover 4.5, turnover interval 1.67 days.
OPD	Monday is the busiest day (310 patients), Saturday the lightest (205); the weekly average is 249 per day.
Discharge	Average 4 hours 10 minutes from the order to departure; bill preparation and insurance approval take 130 minutes.
Records	Completeness 84% to 96%; the lowest is the dating and signature of doctors' notes (84%).
Safety	Hand hygiene 86% against the target of 95%.
Feedback	80% satisfied; the main complaints are the waiting time at billing and the discharge delay; 83.3% of complaints were closed in 48 hours.

10.3 Improvement plan

- Start the final bill during the last day of the stay and send the interim bill to the TPA early.
- Add a billing counter and a doctor on Mondays; use tokens and an appointment system.
- Add a check of the signature and date on the doctors' notes before the file leaves the ward.
- Place hand rub at every bed and give monthly hand hygiene feedback to each ward.
- Review the complaints every month and set the target of 90% closure in 48 hours.

10.4 Limitations

The data is dummy and for one month only, the sample sizes are small, and the hospital is fictitious. A real study needs permission from the hospital, anonymised data and a longer period. The student noted that the learning lies in the method and in the habit of respecting the privacy of every patient.

11. Case Study Exercise: Reducing the Waiting Time in an OPD

A fictitious district hospital OPD sees about 250 patients a day, and patients complain that they wait three hours to see a doctor. The administrator asks the student to find the causes and propose changes.

Observation. For one week the student noted the time of arrival, the time of registration, the time of entering the consultation room and the time of leaving for ten patients each morning. On average, the registration queue took 35 minutes, the wait for the doctor 85 minutes, the consultation 6 minutes, and the wait at the pharmacy 30 minutes. About 60% of patients arrived between 8 and 10 am, since the OPD opened at 8 am, and the doctors came at 9 am.

Analysis. The largest wait (85 minutes) was caused by the mismatch between the arrival time and the doctor timings, and by the lack of a separate queue for follow-up and new patients. The registration wait of 35 minutes was due to two counters for 150 patients in two hours.

Recommendations. Start the doctors' OPD on time, offer appointment slots by phone or by a token for follow-up patients, open a third registration counter from 8 to 10 am, keep a fast-track queue for the elderly, and give the pharmacy the prescription by the doctor's computer so that the medicines are packed while the patient walks across. A target was set to bring the average total time from about 156 minutes to 90 minutes, and to measure it again after one month. The figures are illustrative.

Lessons. Delays are mostly the result of the system and not of the people; measuring the time of each step shows where to act; and a small change at the busiest time gives the largest gain.

12. Mock Practical Exercise: Admission, Billing and Discharge of a Dummy Patient

A mock practical exercise was conducted at the end of the learning phase to check that the student could carry out the main administrative steps of a patient's stay independently within a fixed time. The task was to process a dummy patient from registration to discharge. The steps followed were:

1. Registration: a UHID was created after checking for an existing record, and the consent was recorded.
2. OPD: the patient was given a token and an appointment with the doctor, and the admission was advised.
3. Admission: the admission form was completed, the insurance card was checked and the pre-authorisation request was prepared.
4. Bed allotment: a bed was allotted as per the entitlement and the ward was informed.
5. Daily charges: room, medicine and investigation charges were entered each day.
6. Discharge and bill: the final bill was prepared, the admissible and non-admissible items were separated, and the co-payment was calculated.
7. Records: the file was checked against the completeness list and sent to the MRD.
8. Feedback: a feedback form was given, and one complaint was logged and answered.

13. Sample Checklists Prepared

Admission and claim checklist

- Identity and UHID verified; consent signed.
- Insurance card, ID and policy validity checked; pre-authorisation approved.
- All charges itemised; non-admissible items marked.
- Discharge summary, bills and reports attached to the claim.

NABH readiness checklist (extract)

- Patient rights displayed and staff trained (PRE).
- Hand hygiene audits done and displayed (HIC).
- Medication safety: labelled high-alert drugs, expiry checked (MOM).
- Incident reports collected and reviewed monthly (PSQ).

14. Tools and Resources

The following tools and resources were used during the internship: MS Excel for calculating indicators, bills and charts, MS Word and PowerPoint for the report and the presentation, dummy forms, registers and sample bills of a fictitious hospital, sample hospital information system screens and templates provided by the agency, the official websites of NABH, the National Health Authority and the Ministry of Health and Family Welfare for reference, and regular mentor feedback.

15. Programme at a Glance

The programme was in the domain of Hospital & Healthcare Administration with RKNEXORA PRIVATE LIMITED as the agency. It required a minimum of 120 hours over 28 learning days, organised in three phases: Orientation (Days 1-2), Learning and Practice (Days 3-22) and Project and Review (Days 23-28). Attendance was recorded in the Internship Attendance Log, signed by the supervisor (see Attendance Sheet).

The approximate distribution of the effort across the topics is shown below.

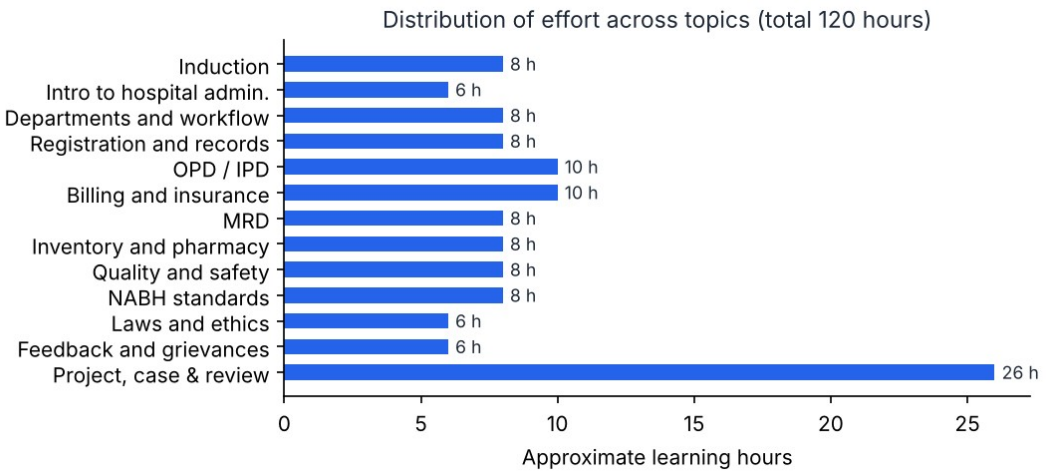


Figure 8: Distribution of learning effort across topics (120 hours)

LEARNING OUTCOME

a. Knowledge Acquired

1. The departments of a hospital and the patient journey.
2. Registration, UHID and record keeping.
3. OPD and IPD management and the calculation of BOR, ALOS and turnover.
4. Itemised billing, package rates and the steps of a cashless insurance claim.
5. Medical records management, filing and confidentiality.
6. Inventory control, reorder level, FEFO and expiry tracking.
7. Patient safety, infection control and audits.
8. The chapters of the NABH standards.
9. Healthcare laws, consent and patient rights.
10. Feedback analysis and grievance handling.

Glossary of Terms Learned

- **UHID** - Unique Health ID of a patient in the hospital.
- **ABHA** - A health account number under the Ayushman Bharat Digital Mission.
- **OPD / IPD** - Outpatient / inpatient department.
- **BOR** - Bed occupancy rate.
- **ALOS** - Average length of stay.
- **Bed turnover** - The number of patients per bed in a period.
- **TPA** - Third Party Administrator for insurance claims.
- **Pre-authorisation** - The insurer's approval before cashless treatment.
- **Co-payment** - The share of the cost paid by the patient.
- **Package rate** - A fixed charge for a set of services.
- **MRD** - Medical Records Department.
- **Consent** - The patient's agreement after being informed.
- **FEFO** - First expiry, first out.
- **Reorder level** - Stock at which a new order is placed.
- **NABH** - National Accreditation Board for Hospitals and Healthcare Providers.
- **HIC** - Hospital infection control.
- **Near miss** - An event that could have caused harm but did not.
- **Grievance** - A complaint by a patient or relative.
- **PM-JAY** - Ayushman Bharat Pradhan Mantri Jan Arogya Yojana.

b. Skills Learned

Technical skills. Patient registration and records; calculating hospital indicators; preparing itemised bills and insurance claims; medical records filing and audits; inventory control with reorder levels; preparing quality audits; understanding NABH standards; and analysing feedback.

Professional skills. Courtesy and empathy; accuracy and attention to detail; confidentiality; coordination between departments; calmness under pressure; clear communication; and time management.

These skills were developed through repeated practice and daily documentation. Working through a dummy patient's stay from registration to discharge built accuracy and coordination, handling dummy complaints built patience and empathy, and explaining the findings to the supervisor built confidence.

c. Observed Attitude and Gained Values

1. **Compassion and respect:** every patient is a person in need and deserves dignity.
2. **Confidentiality:** only dummy data was used, and the duty to protect patient information was understood.
3. **Honesty and accuracy:** correct bills, complete records and truthful reports.
4. **Safety consciousness:** small habits protect lives.
5. **Discipline and punctuality:** regular reporting at fixed timings and daily attendance records.

d. Challenges Faced

Understanding the flow between many departments was handled by drawing the patient journey and timing each step.

Calculating indicators correctly was handled by writing the formula and checking the units (bed-days, discharges).

Understanding insurance terms such as co-payment and non-admissible items was handled by working out sample claims by hand.

Remembering the NABH chapters was handled by making a one-page chart with their codes.

Dealing with a dummy upset relative in a role-play was handled by listening first, apologising for the inconvenience and stating the action.

Managing time between practice, documentation and report writing was handled by dividing work week by week and using review days for corrections.

e. Career Pathways

The knowledge gained supports careers such as Hospital Administrator (junior), Front Office or Patient Relations Executive, Billing and Insurance Executive, TPA and Claims Coordinator, MRD Technician, Quality Coordinator, Pharmacy or Store Assistant, Health Information Management Assistant and Patient Experience Executive. Opportunities exist in government and private hospitals, nursing homes, diagnostic centres, insurance companies and TPAs, health-tech companies and NGOs working in health. Related higher-study options include BBA or MBA in hospital and healthcare management, B.Sc. in hospital administration, MHA, public health programmes, and certificate courses in medical coding, quality management and NABH. A good next step is to learn a hospital information system and to study the NABH and PM-JAY guidelines.

CONCLUSION

The internship in Hospital & Healthcare Administration was a valuable learning experience that integrated academic knowledge with practical activity. The programme achieved its objectives through structured learning, guided practice, assignments, supervisor review and documentation.

The student gained a clear understanding of how a hospital works and how patients, records, billing, supplies, quality and complaints are managed - through the study of departments and workflow, registration, OPD and IPD, billing and claims, MRD, pharmacy, patient safety, NABH, laws and ethics, and grievances - and acquired practical experience by studying the workflow of a fictitious hospital with dummy data. The experience has strengthened the student's employability and provides a foundation for further study and work in healthcare administration.

Reflection

Looking back, the most useful lesson of the internship was that good hospital administration is mostly invisible: when the registration, bed, record, medicine and bill are right, the patient notices only the care. The timing of each step of the discharge showed that delays come from the system, and the insurance worked example showed how a small missing document can hold up a large claim. Practising calm replies to a dummy complaint showed that listening is the first step of service, and the importance of confidentiality was clear in every task.

Objectives and Achievement

The objective of understanding departments and workflow was achieved through the department chart and the patient journey. The objective of registration, OPD and IPD was achieved through the registration form, the BOR and ALOS calculation and the OPD chart. The objective of billing and records was achieved through the itemised bill, the claim and the records audit. The objective of inventory, quality and NABH was achieved through the reorder level, the safety audit and the NABH checklist. The objective of laws, ethics and grievances was achieved through the consent checklist and the analysis of 120 feedback forms.

Future Scope

The knowledge gained can support further study of hospital and health management and careers in healthcare administration. With the growth of health insurance, Ayushman Bharat, digital health records and private hospitals in Bihar and across India, there is a continuing need for trained administrators who combine accuracy with compassion.

Suggestions for Improvement of the Programme

1. Provide practice with a hospital information system along with MS Excel.
2. Include a short module on medical coding and insurance claim documentation.
3. Add a visit to a hospital or a simulated hospital for observation, with proper permission.
4. Continue regular mentor feedback and progress reviews.
5. Encourage students to maintain detailed daily documentation.

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8. Government of India. The Clinical Establishments (Registration and Regulation) Act, 2010; The Consumer Protection Act, 2019; The Digital Personal Data Protection Act, 2023.
9. Ministry of Environment, Forest and Climate Change. Bio-Medical Waste Management Rules, 2016.
10. RK NEXORA internship programme handbook.

ATTENDANCE SHEET

The Internship Attendance Log issued by the IPO, with the supervisor's signature for each day, is to be attached on this page.

[Attach the signed Internship Attendance Log here]